

UNIVERSITY MEDICAL CENTER
HEMATOLOGY-ONCOLOGY OUTPATIENT CLINIC
FOLLOW-UP NOTE

Date: 14.03.2024

Patient: Roger Cook

DOB: 16.02.1976

MRN: AML059

SUBJECTIVE:

47 y/o male with favorable-risk AML (bZIP inframe CEBPA mutation, Dx date: 07.07.2023) s/p CPX-351 induction+consolidation returns for routine f/u. Pt reports excellent energy, returned to work as software engineer full-time. No fever, night sweats, bleeding, or bruising. Appetite good, weight stable at 82kg. Some mild peripheral neuropathy in fingertips (grade 1) persisting from treatment.

OBJECTIVE:

VS: T 36.8°C, BP 128/78, HR 72, O2sat 98% RA

Gen: Well-appearing male in NAD

HEENT: PERRL, no lymphadenopathy

CV: RRR, no m/r/g

Pulm: CTAB

Abd: Soft, NT/ND, no HSM

Ext: No edema, no rash

Neuro: Mild decreased sensation fingertips b/l

LABS (today):

WBC 6.2 (ANC 3.8), Hgb 13.4, Plt 245

Cr 0.9, BUN 16, AST 22, ALT 28, LDH 156

Flow cytometry: No evidence residual leukemia

ASSESSMENT/PLAN:

AML in continued complete remission - excellent response to CPX-351 after 8 months. Will continue q3month monitoring with CBC/diff, CMP, bone marrow q6months. Discussed allogeneic SCT - pt defers given excellent response and young age. Continue quarterly visits. Next appointment June 2024.

Dr. Kevin Thompson, MD